This Patient Group Direction (PGD)is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the administration of Fexofenadine and/or Dymista for the treatment of Allergic Rhinitis

Patient Group Direction, version 007, issued 30 September 2026. Supersedes Version 006, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Summary of NICE CKS Guidelines for Allergic Rhinitis

Overview

- **Allergic rhinitis** is an **IgE-mediated inflammatory condition** of the nasal mucosa triggered by allergens (e.g. pollen, dust mites, animal dander).
- Can be **seasonal** (e.g. hay fever), **perennial**, or **occupational**.
- Often associated with **asthma**, **eczema**, and **conjunctivitis**.

Symptoms

- Sneezing
- Nasal congestion
- Rhinorrhoea (runny nose)

- Nasal itching
- Postnasal drip
- Itchy/watery eyes (if allergic conjunctivitis also present)

Assessment

- Diagnosis is clinical based on symptom pattern and triggers.
- Determine if:
    - **Intermittent** (<4 days/week or <4 weeks) or **persistent** (≥4 days/week and ≥4 weeks)
    - **Mild** (normal sleep/daily activities) or **moderate/severe** (disturbed sleep/impacts activities)

Consider:

- Trigger history (seasonal vs year-round)
- Coexisting asthma or eczema
- Impact on quality of life

Management

1. Allergen Avoidance and Education

- Limit exposure to known triggers (e.g. close windows, use pollen filters).
- Saline nasal irrigation can be helpful.

2. Pharmacological Treatment (Stepwise Approach)

Step	Treatment	Notes
First-line	**Non-sedating oral antihistamines** (e.g. loratadine, cetirizine)	Good for sneezing, runny nose, itchy eyes
Add or switch to	**Intranasal corticosteroid spray** (e.g. fluticasone, beclometasone)	Most effective monotherapy for nasal symptoms
Additional options	- **Antihistamine eye drops** (e.g. olopatadine) for ocular symptoms	

- **Intranasal antihistamines** (e.g. azelastine)
- **Decongestants** (short-term only, max 7 days) | Combine with nasal steroids if needed |
- Consider **combination sprays** (e.g. fluticasone + azelastine) for moderate/severe symptoms.

3. Immunotherapy (Specialist Only)

- Consider for confirmed **IgE-mediated allergy** (e.g. pollen) with **persistent symptoms** not controlled by medications.
- Administered **subcutaneously or sublingually**.

When to Refer

- Poor response to optimal treatment
- Diagnostic uncertainty
- Suspected structural abnormalities (e.g. nasal polyps)
- Consider referral for **allergy testing** or **immunotherapy assessment**

Monitoring and Follow-Up

- Review symptom control and medication adherence.
- Assess for complications: sinusitis, sleep disturbances, or asthma flare-ups.
- Adjust treatment based on response and seasonal variation.

Patient Group Direction

for the supply/administration of

Fexofenadine 120mg tablets

for the treatment of

Allergic Rhinitis

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	• Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required ● All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service

Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Fexofenadine is indicated for the symptomatic relief of allergic rhinitis, including seasonal hay fever, in adults and adolescents aged 12 years and over.
Inclusion criteria	- Individuals aged 12 years and over with symptoms consistent with allergic rhinitis. - Informed consent obtained. - No contraindications to fexofenadine. - Previous diagnosis of allergic rhinitis or recurrence of known symptoms.
Exclusion criteria	- Known hypersensitivity to fexofenadine or any component of the formulation. - Severe renal or hepatic impairment. - Pregnancy or breastfeeding. - Under 12 years of age.
Cautions	- Caution in individuals with a history of cardiovascular disease. - Advise avoidance of alcohol and other sedating antihistamines. - If symptoms persist beyond one month of regular use or worsen, refer to a healthcare provider. - Ensure patients are aware this is a non-sedating antihistamine but occasional drowsiness may still occur.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Fexofenadine 120mg tablets
Legal category	P (Pharmacy medicine) for fexofenadine 120 mg as Allevia; generic fexofenadine 120 mg tablets remain POM, and this PGD is the legal authority for those. Where the P licence is narrower than this PGD, the P licence governs.
Storage	Store below 25°C in a dry place.
Route/method of administration	Oral administration with water before food.
Dose and frequency	120 mg once daily, the licensed dose for allergic rhinitis in adults and children aged 12 years and over. The 180 mg strength is licensed for chronic idiopathic urticaria only and is not supplied under this PGD.
Quantity to be administered and/or supplied	Up to 30 tablets (1 month supply).
Maximum or minimum treatment period	As required for symptom control, typically seasonal use.

Adverse effects	Headache, drowsiness, dizziness, nausea. Rare: hypersensitivity reactions. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply, dose, form, route and quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken ● that the medicine was supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen rapidly or significantly, persist beyond one month of regular use despite treatment, or they become systemically very unwell.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- NICE CKS Guidance
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines BNF (British National Formulary) | NICE
- Summary of Product Characteristics

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
30/9/26		31/7/27	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date

Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	Change details	Date
001	Development & issue of new PGD	25th June 2025

Patient Group Direction

for the supply/administration of

Dymista nasal spray, suspension: azelastine hydrochloride 137 micrograms and fluticasone propionate 50 micrograms per actuation, 23 g bottle delivering 120 actuations (Viatris)

for the treatment of

Allergic Rhinitis

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	• Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required ● All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service

Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Dymista is indicated for the relief of moderate to severe seasonal or perennial allergic rhinitis in adults and adolescents aged 12 years and over, where monotherapy with either intranasal antihistamine or corticosteroid is not sufficient.
Inclusion criteria	- Adults and adolescents aged 12 years and over. - Symptoms of moderate to severe allergic rhinitis requiring dual therapy. - Informed consent obtained. - No contraindications to Dymista or its components.
Exclusion criteria	- Known hypersensitivity to azelastine, fluticasone, or any excipients. - Untreated fungal, bacterial, or viral nasal infection. - Recent nasal surgery or trauma. - Pregnancy or breastfeeding. Refer to the GP. - Children under 12 years of age.
Cautions	- Advise on correct nasal spray technique to optimise efficacy and reduce side effects. - Caution in individuals with hepatic impairment, glaucoma, or tuberculosis. - Reassess need for continued treatment if symptoms persist beyond 4 weeks. - Counsel on possible side effects and need for ongoing review if used long-term.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Dymista nasal spray, suspension: azelastine hydrochloride 137 micrograms and fluticasone propionate 50 micrograms per actuation, 23 g bottle delivering 120 actuations (Viatris)
Legal category	POM
Storage	Store below 25°C. Do not refrigerate or freeze.
Route/method of administration	Intranasal spray, one spray in each nostril twice daily.
Dose and frequency	One spray in each nostril twice daily (morning and evening).
Quantity to be administered and/or supplied	One bottle (23 g), approx. 120 sprays.
Maximum or minimum treatment period	Use as required; assess effectiveness after 2 to 4 weeks. One bottle per supply; reassess the need for continued treatment before any repeat supply and if symptoms persist beyond 4 weeks.

Adverse effects	Nasal irritation, headache, epistaxis (nosebleeds), bitter taste, somnolence. Rare: hypersensitivity reactions. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply, dose, form, route and quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken ● that the medicine was supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen rapidly or significantly, do not improve after 2 to 4 weeks of regular use, or they become systemically very unwell.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
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Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date

Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	Change details	Date
001	Development & issue of new PGD	25th June 2025

Version and change record

Version: v007

Issued: 30 September 2026

Supersedes: Version 006, 14 September 2026

Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records

Version: v006

Issued: 14 September 2026

Supersedes: Version 005, 11 September 2026

Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.

Previous version records

Version: v005

Issued: 11 September 2026

Supersedes: Version 004, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Decision 57: pregnancy and breastfeeding are added to the Dymista arm's exclusion criteria, with referral to the GP, so both arms of this PGD now exclude them.

Decision 58: the fexofenadine review and referral point is one month of regular use, stated in the cautions row and in the follow-up advice row in place of 7 days, matching the 30-tablet one-month supply.

Decisions of the authorising signatories, 11 September 2026: the questions raised by the reviews of 10 and 11 September were put to the Medical Director and answered; each answer is written into this document above and, where the answer set a rule, into the consultation tool. Text drafted from a source for the Head Pharmacist to confirm is marked as such in the change lines.

Previous version records

Version: v004

Issued: 11 September 2026

Supersedes: Version 003, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Fexofenadine follow-up advice: the 3 to 4 week template line is replaced with the arm's own 7-day referral point from the cautions row.

Dymista follow-up advice: the 3 to 4 week template line is replaced with the arm's own 2 to 4 week effectiveness review.

Fexofenadine legal category: narration about why the P arm is included was removed; the P-licence-governs rule stays.

Dymista treatment-period row: one bottle per supply and reassessment before any repeat supply are stated, matching the 4-week caution.

Records row: spacing typo in the under-18 retention bullet corrected.

Records row: the two run-together bullets were separated and punctuated (date of supply, dose, form, route and quantity supplied; adverse reactions and actions taken; supplied via PGD).

Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v003

Issued: 11 September 2026

Supersedes: Version 002, 10 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Fexofenadine strength corrected to 120 mg, the strength licensed for allergic rhinitis. Version 002 named 180 mg, which is licensed only for chronic idiopathic urticaria, without declaring off-label use. Legal category stated correctly: 120 mg is P as Allevia and POM as the generic.

Dymista is named in full with its form, both active ingredients and strengths, pack size and marketing authorisation holder.

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Previous version records

Version: v002

Issued: 10 September 2026

Supersedes: Version 001, 25 June 2025

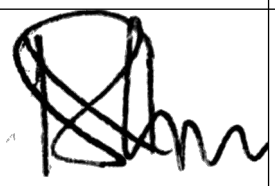
Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded. The previous version carried three different dates for one version (issued 25 June 2025, signed 25 August 2025, valid from 25 August 2026) and empty validity cells.

Signed on behalf of Get Real Health

Version v007, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.